

Military Disability Nexus — Community Q&A

Optimisation Pack

Schema Templates + Rewritten Answers + Page Metadata

Prepared for militarydisabilitynexus.com/community

PART 1 — QAPage JSON-LD Schema Template

Paste this inside a `<script type="application/ld+json">` tag in the `<head>` of **every individual question page**. Replace the placeholder values for each question.

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  "description": "[META DESCRIPTION — see Part 2 for each question]",
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```

Implementation Notes for Your Developer

- The `acceptedAnswer.text` field must be **plain text only** — strip all HTML and markdown before inserting
- `dateCreated` should reflect the actual date the question/answer was posted
- Replace `[SME FULL NAME]` and `[jobTitle]` with your clinician's real credentials — this is critical for Google's E-E-A-T scoring
- Add an `id="expert-answer"` anchor to the expert answer div in the HTML so the URL fragment works
- The `upvoteCount` on the answer should update dynamically to match your vote count

PART 2 — All 6 Questions: Optimised Titles, Meta Descriptions

& Rewritten Answers

Q1: Nerve Issues / Numbness and Tingling in Arm

Current page title: I have numbness and tingling down my arm. How does the VA rate nerve issues?

Optimised page title (≤60 chars): VA Rating for Arm Numbness & Tingling: Cervical Radiculopathy Guide

Meta description (≤155 chars): The VA rates arm numbness as a peripheral nerve condition. Ratings range from 10%-60%+ based on severity and which nerve is affected. Expert answer inside.

Optimised URL slug (if you ever restructure): va-disability-rating-arm-numbness-tingling-cervical-radiculopathy

REWRITTEN EXPERT ANSWER

[Direct Answer — what AI engines pull first]

Numbness and tingling down the arm is most commonly rated by the VA as cervical radiculopathy or an upper extremity peripheral nerve condition under 38 CFR Part 4. The rating is based on the severity of nerve impairment and which specific nerve is affected — not on MRI findings alone.

How the VA determines the rating

The VA evaluates nerve conditions using a scale of incomplete to complete paralysis:

- **Mild incomplete paralysis** → 10%–20%
- **Moderate incomplete paralysis** → 20%–40%
- **Severe incomplete paralysis** → 40%–60%
- **Complete paralysis** → 60%+ depending on the nerve and limb

Ratings also differ based on whether it's your **dominant (major)** or **non-dominant (minor)** arm — the dominant arm is rated slightly higher because the functional loss is greater.

What the C&P examiner actually tests

During your Compensation & Pension exam, the examiner will assess muscle strength (using

a 0–5 scale), grip strength, reflexes, sensation, and any visible muscle wasting. The key phrase in your exam report is “functional impairment” — the more clearly your impairment is documented in those specific terms, the better your rating outcome.

The secondary connection most veterans miss

If your arm symptoms are caused by a service-connected neck or spine condition (such as a cervical disc herniation), the nerve condition can be rated **separately** as a secondary condition — meaning you get a rating for both the neck and the nerve damage on top of it. This is one of the most commonly missed rating opportunities in orthopedic claims.

If you have a neck condition already rated by the VA and you’re experiencing arm numbness, a Nexus Letter documenting the secondary connection between your cervical spine and the nerve impairment can be the difference between a single rating and a combined rating that accurately reflects your full disability picture. [Learn more about secondary Nexus Letters →](#)

Veterans also asked:

- [My migraines seem to be triggered by my tinnitus — will VA consider that as a secondary connection?](#)
- [How do I make sure I don’t undersell my symptoms at a C&P exam?](#)

Q2: Migraines Triggered by Tinnitus — Secondary Connection

Current page title: My migraines seem to be triggered by my tinnitus, will VA consider that?

Optimised page title (≤60 chars): Can Tinnitus Cause Migraines? VA Secondary Service Connection Explained

Meta description (≤155 chars): Yes — if tinnitus triggers your migraines, you may be able to claim migraines as a secondary condition to your tinnitus. Expert VA answer here.

Optimised URL slug: va-secondary-connection-tinnitus-migraines

REWRITTEN EXPERT ANSWER (*currently unanswered — this is a new answer*)

[Direct Answer]

Yes, the VA can consider migraines as a secondary service-connected condition to your tinnitus — but you need medical evidence establishing that link, not just your personal

observation. If you already have tinnitus rated by the VA, migraines could be rated separately and on top of it.

How secondary service connection works

Secondary service connection means that Condition B (migraines) was caused or aggravated by Condition A (tinnitus), which is already service-connected. The VA requires a medical nexus — a clinician's documented opinion stating that it is "at least as likely as not" that the tinnitus is causing or contributing to the migraines.

Your personal statement that tinnitus seems to trigger your migraines is valuable, but it is not enough on its own. You need a treating clinician or an independent medical examiner to provide that nexus opinion in writing.

What the medical literature supports

There is a recognised clinical relationship between chronic tinnitus and migraine. Both conditions involve central sensitisation — an abnormal amplification of pain and sensory signals in the nervous system. The constant auditory stress of tinnitus can trigger or worsen migraine attacks in susceptible individuals. This is a defensible connection that a qualified clinician can support in a Nexus Letter.

What your rating could look like

Tinnitus is capped at 10% (Diagnostic Code 6260). Migraines, however, are rated separately under DC 8100 and can reach 50% if you have prostrating attacks occurring very frequently. If both are service-connected, they combine — meaning your total compensation increases significantly beyond the 10% tinnitus cap.

What to do next

Document in your medical records that you experience migraines and that tinnitus appears to be a trigger. Then request an Independent Medical Opinion (Nexus Letter) from a qualified clinician who can formally establish the secondary connection. Without that written nexus, the VA will not grant secondary service connection regardless of how clear the relationship seems to you. [Learn more about Nexus Letters for secondary connections →](#)

Veterans also asked:

- [My tinnitus is constant — can I get more than 10%?](#)
- [My migraines wipe me out for hours — does that count as a prostrating attack?](#)

Q3: Tinnitus Rating Above 10%

Current page title: My tinnitus is constant and significantly affects my health. Can I get more than 10% rating?

Optimised page title (≤60 chars): VA Tinnitus Rating: Why 10% Is the Cap and How to Get More

Meta description (≤155 chars): The VA caps tinnitus at 10% under DC 6260, no matter how severe. But secondary conditions like sleep apnea or migraines can significantly increase total compensation.

Optimised URL slug: va-tinnitus-rating-above-10-percent

REWRITTEN EXPERT ANSWER

[Direct Answer]

No — under current VA regulations, the maximum schedular rating for tinnitus itself is 10%, regardless of severity. This is set by Diagnostic Code 6260, and it applies whether your tinnitus affects one ear or both, is constant or intermittent, or severely impacts your sleep and concentration. The 10% is a hard regulatory ceiling for the primary condition.

Why veterans with severe tinnitus still get just 10%

The VA's rating schedule for tinnitus was written with a flat rate rather than a severity scale. That means a veteran with barely noticeable ringing and one who is completely unable to sleep because of constant bilateral tinnitus both receive the same 10% rating for the condition itself. This frustrates many veterans, and understandably so — but the path to higher total compensation isn't through fighting the 10% cap. It's through documenting the secondary conditions tinnitus causes.

How to increase your total rating when tinnitus is severe

The conditions most commonly caused or worsened by chronic tinnitus include:

- **Obstructive Sleep Apnea** — sleep disruption from tinnitus can be documented as a secondary condition (rated at 0%, 30%, 50%, or 100%)
- **Migraines** — auditory stress from tinnitus is a recognised migraine trigger; rated separately under DC 8100 (up to 50%)
- **Depression or Anxiety** — chronic tinnitus is strongly associated with psychiatric conditions, which can be rated under mental health diagnostic codes (up to 100%)
- **Hearing Loss** — frequently co-occurs with tinnitus and rated separately under its own diagnostic code

Each of these, if service-connected as secondary to your tinnitus, adds to your combined disability rating on top of the 10% you already receive. A veteran with tinnitus (10%) + secondary sleep apnea (50%) + secondary migraines (30%) ends up with a combined rating well above what the tinnitus rating alone would suggest.

What you need to make this work

Secondary service connection requires a medical nexus — a written clinical opinion establishing that your secondary condition was caused or aggravated by your service-connected tinnitus. This is exactly what a Nexus Letter provides. Without it, the VA will deny the secondary claim regardless of how clear the connection seems clinically. [Explore Nexus Letters for tinnitus secondary conditions →](#)

Veterans also asked:

- [My migraines seem to be triggered by my tinnitus — will VA consider that?](#)
 - [My PTSD is getting worse — why is the VA keeping me at 30%?](#)
-

Q4: Prostrating Migraine Attacks

Current page title: My migraines wipe me out for hours does that count as a 'prostrating' attack?

Optimised page title (≤60 chars): What Is a Prostrating Migraine? VA Rating Criteria Explained

Meta description (≤155 chars): If your migraines force you to stop everything and lie down for hours, they likely qualify as prostrating under VA criteria — which directly affects your rating.

Optimised URL slug: what-is-a-prostrating-migraine-va-rating

REWRITTEN EXPERT ANSWER

[Direct Answer]

Yes — if your migraines force you to stop all activity and lie down for hours, that almost certainly meets the VA's definition of a prostrating attack. Whether the VA formally recognises it depends on how well that functional impact is documented in your medical records and how clearly you describe it at your C&P exam.

What "prostrating" actually means to the VA

The VA does not have a rigid clinical checklist for “prostrating,” but in practice it means an attack severe enough to force complete cessation of activity. If any of the following apply, your migraine is likely prostrating:

- You have to lie down in a dark, quiet room
- You cannot continue working, driving, or performing normal daily tasks
- You experience severe pain, nausea, vomiting, photophobia (light sensitivity), or phonophobia (sound sensitivity)
- The attack lasts at least several hours
- You are essentially non-functional for the duration of the episode

Why this matters: it directly determines your rating

Migraine ratings under Diagnostic Code 8100 are built around the frequency and severity of prostrating attacks:

- **Less frequent prostrating attacks** → 10%
- **Prostrating attacks once a month on average** → 30%
- **Prostrating attacks occurring very frequently, with economic inadaptability** → 50%

The 50% rating is the schedular maximum. “Economic inadaptability” means the migraines significantly interfere with your ability to hold and maintain employment.

The documentation problem most veterans face

Many veterans describe their migraines to doctors as “bad headaches” without conveying the full functional impact. If your medical records say “headache, treated with ibuprofen” rather than “incapacitating migraine requiring withdrawal from activity for 4–6 hours,” the VA will not have the evidence needed to rate them as prostrating — even if that’s exactly what’s happening.

What you describe at your C&P exam and what’s in your treatment records need to match, and both need to clearly reflect the functional impairment. Keeping a migraine diary — logging onset, duration, symptoms, and what you couldn’t do during each attack — is one of the most effective ways to build that evidence pattern before your exam.

If you are approaching a C&P exam for migraines, our coaching can help you communicate the full scope of your symptoms accurately. [Learn about C&P Exam Coaching →](#)

Veterans also asked:

- [My migraines seem to be triggered by my tinnitus — will VA consider that?](#)

- [How do I make sure I don't undersell my symptoms at my C&P exam?](#)
-

Q5: C&P Exam — Not Underselling Symptoms

Current page title: I have a C&P exam coming up — how do I make sure I don't undersell my symptoms?

Optimised page title (≤60 chars): C&P Exam Tips: How to Accurately Describe Symptoms Without Underselling

Meta description (≤155 chars): Veterans routinely undersell symptoms at C&P exams out of habit. Here's exactly what to say, what not to say, and how to describe your worst days accurately.

Optimised URL slug: cp-exam-tips-dont-undersell-symptoms-va

REWRITTEN EXPERT ANSWER

[Direct Answer]

The most important mindset shift going into a C&P exam: describe your worst days, not your average days, and never say “I’m fine” or “I manage okay.” The VA rates you on functional impairment — what your condition prevents you from doing — not on a diagnosis or a clinician’s sympathy. If the examiner doesn’t hear it, it doesn’t exist.

The habits that cause veterans to undersell

Military culture trains you to minimise. You’ve been conditioned to push through pain, report ready, and not complain. That instinct will cost you at a C&P exam. Common mistakes include:

- Answering “how’s your pain today?” with today’s actual number instead of your worst
- Saying “I get around okay” when you’ve modified how you move to avoid pain
- Not mentioning secondary effects like sleep loss, depression, or relationship strain
- Forgetting to describe how symptoms affect your ability to work specifically

What to prepare and say

Before the exam, write down a concrete account of your worst episode in the past 12 months. For every symptom, know:

1. **Frequency** — how often does it occur?
2. **Duration** — how long does each episode last?
3. **Severity at its worst** — what is it like at its worst, not its average?
4. **Functional impact** — what can't you do during or after an episode? (Can't drive, can't sit at a desk, had to call out of work, had to cancel family commitments)
5. **Flare-up triggers** — what makes it worse? (Stress, physical activity, weather, noise)

What not to say

Avoid minimising language: "it's not that bad," "I've learned to live with it," "most days I'm okay." These are all flags the VA examiner may record as evidence of mild impairment. If most days you *are* okay but certain days you are not functional, say exactly that: "On a good day I can manage X, but I have approximately [N] bad days per month where I am unable to [specific activity]."

The documentation gap

If your treatment records don't reflect what you tell the examiner, the VA will weight the records over your testimony. Before your exam, review what your doctors have actually written about your condition. If there are gaps — months where you didn't seek treatment because you were trying to cope — write a personal statement explaining that gap. Gap in records ≠ gap in symptoms.

If you want structured preparation before your C&P exam, we offer coaching sessions specifically designed to help veterans communicate their symptoms accurately without overstating or underselling. [Learn about C&P Exam Coaching →](#)

Veterans also asked:

- [My PTSD is getting worse — why is the VA still keeping me at 30%?](#)
- [My migraines wipe me out for hours — does that count as a prostrating attack?](#)

Q6: PTSD Rating Stuck at 30%

Current page title: My PTSD feels like it's getting worse so why is the VA still keeping me at 30%?

Optimised page title (≤60 chars): PTSD Rating Stuck at 30%? Why the VA Won't Increase It — and What to Do

Meta description (≤155 chars): The VA rates PTSD on functional impact, not how you

feel. If your records don't show worsening impairment, your rating won't move. Here's how to fix that.

Optimised URL slug: `ptsd-rating-stuck-30-percent-va-wont-increase`

REWRITTEN EXPERT ANSWER

[Direct Answer]

If your PTSD rating is stuck at 30% despite your symptoms worsening, it almost always comes down to one thing: your medical records are not documenting the functional impact of your deterioration. The VA does not rate PTSD based on how severe your symptoms feel to you — they rate based on how clearly those symptoms are documented as impairing your ability to work, maintain relationships, and carry out daily responsibilities.

How VA rating criteria work for PTSD

PTSD is rated under the General Rating Formula for Mental Disorders (38 CFR § 4.130). The rating levels and what separates them:

- **30%** — occasional decrease in work efficiency, difficulty with complex tasks, periods of inability to perform occupational and social functioning
- **50%** — reduced reliability and productivity, flattened affect, panic attacks more than once per week, difficulty with understanding complex commands, impaired judgment, disturbance of motivation and mood
- **70%** — deficiencies in most areas including work, school, family relations, judgement, thinking, and mood; near-continuous panic or depression; inability to function independently
- **100%** — total occupational and social impairment

To move from 30% to 50% or higher, your medical records need to show consistent evidence of impairment in occupational and social functioning — not just symptom descriptions, but real-world consequences. Things like: missed work, inability to maintain employment, strained or broken relationships, withdrawal from social activities, panic attacks with documented frequency.

Why documentation is the core problem

Veterans often present their best self to treatment providers. They minimise. They say “I’m managing” because that’s the trained response. When a clinician writes “veteran reports managing symptoms with coping strategies,” that note works against a higher rating — even

if the reality at home is very different.

Many veterans also only seek treatment during acute flare-ups, which creates a medical record that looks like intermittent mild symptoms rather than a worsening chronic condition. The VA sees the records, not the lived experience.

What actually moves the rating up

- **Consistent, detailed documentation** from your mental health provider describing functional impairment — not just symptoms. Your therapist or psychiatrist should be writing about the real-world impact: missed appointments, employment difficulties, relationship problems, sleep disruption affecting daily function.
- **A Buddy Statement** (lay statement under 38 CFR § 3.303) from a spouse, family member, or friend who can describe what they observe at home — the isolation, the mood changes, the days you can't get out of bed. This is sworn evidence the VA must consider.
- **A formal PTSD Nexus Letter or IMO** from a clinician — particularly useful if you are seeking a rating increase based on aggravation or if you're filing a supplemental claim. An independent clinician who reviews your full history can write a clinical opinion establishing the severity of your current functional impairment in terms that map directly to the VA's rating criteria.
- **An updated C&P exam** — if it has been 12+ months since your last exam and your condition has worsened, you can request a reexamination by submitting new evidence of worsening with a supplemental claim.

The bottom line

A worsening rating is not automatic — it requires a paper trail that shows the VA what is happening to your life, not just your symptoms. If your records currently don't reflect the severity of your impairment, that is the gap to close before filing for an increase. [Book a free call to discuss your PTSD claim →](#)

Veterans also asked:

- [How do I make sure I don't undersell my symptoms at my C&P exam?](#)
- [My tinnitus is constant — can I get more than 10%?](#)

PART 3 — Community Hub Page Optimisation (militarydisabilitynexus.com/community)

Current page title: Community Q and A | VA Benefits Questions | Military Disability Nexus

Optimised page title: VA Benefits Q&A Forum – Expert Answers for Veterans |
Military Disability Nexus

Meta description: Free expert answers to VA disability claim questions – from clinicians, not Reddit. Search by condition: PTSD, tinnitus, sleep apnea, migraines, nexus letters and more.

H1 tag (currently “Community Q and A”): Change to: VA Benefits Questions – Answered by Medical Experts, Free

Add to the page intro (currently just one line):

Ask any question about your VA disability claim and get an answer from our clinical team — at no cost. Unlike Reddit or Facebook groups, every Expert Answer here comes from a licensed clinician with direct VA claims experience. No guesswork. No conflicting opinions. Just accurate guidance.

PART 4 — 15 High-Priority Questions to Pre-Seed

Add these as questions with full expert answers before anyone asks them. These match high-volume veteran search queries.

1. What is a nexus letter and do I actually need one for my VA claim?
2. How do I claim sleep apnea as secondary to PTSD?
3. Can GERD be service connected secondary to stress or PTSD?
4. What is VA Individual Unemployability (TDIU) and how do I qualify?
5. Can the VA reduce my disability rating once it's been assigned?
6. How does VA math work — why doesn't 50% + 50% equal 100%?
7. What is the difference between a DBQ and a Nexus Letter?
8. What documents do I need to file a VA disability claim from scratch?
9. My C&P exam report was wrong — what can I do?
10. How do I claim hypertension as service connected?
11. What is a buddy statement and how do I write one?
12. Can I claim hearing loss and tinnitus separately?

13. What does "at least as likely as not" mean in a VA claim?
14. How long does a VA claim take, and what can I do to speed it up?
15. What is the PACT Act and does it affect my existing claim?

End of Optimisation Pack Military Disability Nexus | militarydisabilitynexus.com